most common streptococcal diseases. Tonsillitis is more common in older children and adults. The pathogen may spread from throat to the surrounding tissues leading to suppurative (pus – formation) complication such as cervical adenitis (inflammation of a lymph node in the neck) otitis media (inflammation of middle ear), quinsy (ulcers of tonsils) Ludwig's angina (purulent inflammation around the sub maxillary glands) mastoiditis (inflammation of mastoid process).

b. Scarlet fever: The disease consists of combination of sore throat and a generalized erythematous (redness of skin or mucous membranes) rash.

2. Skin infections

a. Erysipeals: It is an acute spreading lesion. The skin shows massive brawny oedema with erythema it is seen in elderly persons or elders.

b. Impetigo: (Streptococcal pyoderma)

It is a skin infection that occurs most often in young children. It consists of superficial blisters that break down and eroded areas whose surface is covered with pus. It is the main cause leading to acute glomerulonephritis in children.

c. Necrotizing fasciitis: It is an invasive, infection characterized by inflammation and necrosis of the skin, subcutaneous fat and fascia. It is a life-threatening infection.



The strain which cause necrotizing fasciitis to have been named as "Flesh eating bacteria or" killer bacteria.

3. Streptococcal toxic shock syndrome

Streptococcal pyrogenic exotoxin leads to streptococcal toxic shock syndrome (TSS). It is a condition in which the entire organ system collapses, leading to death.

4. Genital infections

Streptococcus pyogenes is an important cause of puerperal sepsis or child bed fever (infection occur when bacteria infect the uterus following child birth)

5. Deep infection

Streptococcus pyogenes may cause pyaemia (blood poisoning characterized by pus forming pathogens in the blood) septicemia (A condition in which bacteria circulate and actively multiply in the bloodstream) abscess in internal organs such as brain, lung, liver and kidney.

Non – Suppurative Complication

Streptococcus pyogenes infections are sometimes followed by two important non – suppurative complications which are, acute rheumatic fever and acute glomerulonephritis. These complications occur 1–4 weeks after the acute infection and it is believed to be the result of hypersensitivity to some streptococcal components.

1. Rheumatic fever

It is often preceded by sore throat and most serious complication of haemolytic streptococcal infection. The mechanism by which *Streptococci* produce rheumatic fever is still not clear. A common cross – reacting antigen exist in some group A streptococci and heart, therefore, antibodies produced in response to the

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